BETA · FUNDRAISING INTELLIGENCE REPORT

What's Working.
How to Get More of It.

A results-focused analysis of conversion patterns, high-performing models, and the specific levers that generate qualified major giving pipeline — drawn from 3,098 prospects and $46.1 billion in identified wealth across the South Division.

DIVISION	FOCUS	DATA SOURCE	CLASSIFICATION
South Division — California	Results Generation & Replication	Prysm Platform — Feb 2026	Internal — Leadership Briefing

461	$6.6B	$4.32M	5.2×
QUALIFIED MAJOR GIVING	QUALIFIED MG NET WORTH	REALIZED REVENUE (INITIAL)	BEST VS. AVG CONVERSION

SECTION 01

Results Achieved — The Current State

Before asking how to generate more, it's critical to understand exactly what the current model is producing — and which parts of the system are driving the majority of results.

QUALIFIED TO MAJOR GIVING	QUALIFIED MG NET WORTH	REALIZED REVENUE	SYSTEM-WIDE PIPELINE
461	**$6.6B**	**$4.32M**	**$46.1B**
From 3,098 total prospects — a 14.9% system-wide conversion rate with massive variance by ministry	Combined net worth of all prospects who reached major giving qualification stage	Actual revenue from initial Prysm deployment — with 448% engagement increase and 305% first-time MG gift increase	Total identified net worth — 85% of which ($39.5B) has not yet converted to qualified major giving

The South Division has generated **461 qualified major giving prospects** worth $6.6 billion in aggregate net worth, with an initial realized revenue of $4.32 million from the Prysm platform deployment. These results include a 305% increase in first-time major gifts and a 448% increase in engagement. But the results are not evenly distributed — three ministries generate 66% of all qualifications, and two development officers alone account for 30% of the total.

The question this report answers: **What exactly is producing these results, and how do we systematically generate more of them?**

SECTION 02

Where Results Come From

Three dimensions drive qualified major giving output: which ministry, which contact method, and which referral source. Each dimension has a clear winner — and a clear pattern for replication.

EXHIBIT A — THE CONVERSION SCOREBOARD

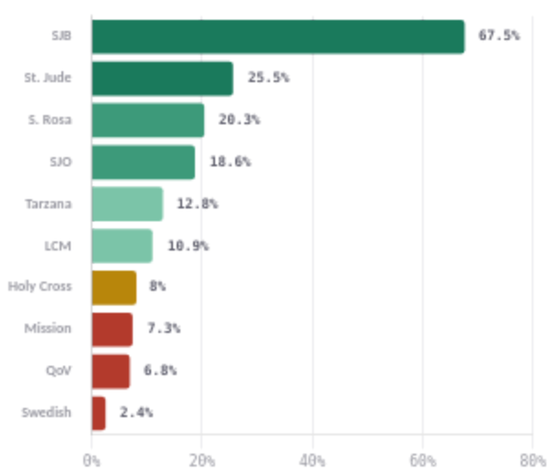
Ministry Results Ranked by Output

Major giving qualifications · Conversion rate · Qualified net worth

MINISTRY	PROSPECTS	MG QUAL	MG RATE	QUALIFIED NW	AVG MG NW	PROFILE
St. Joseph Burbank	206	**139**	**67.5%**	$1,449M	$10.4M	Model
St. Joseph Orange	532	99	18.6%	$1,632M	$16.5M	Mature
St. Jude Medical Center	251	64	25.5%	$542M	$8.5M	Mature
Little Company of Mary	522	57	10.9%	$1,199M	$21.0M	Opportunity
Santa Rosa Memorial	143	29	20.3%	$466M	$16.1M	Mature
Mission Hospital	343	25	7.3%	$599M	$24.0M	Underperform
Tarzana Medical Center	141	18	12.8%	$385M	$21.4M	Growing
Holy Cross Medical Center	188	15	8.0%	$145M	$9.7M	Underperform
Swedish	368	9	2.4%	$171M	$19.0M	Early Stage

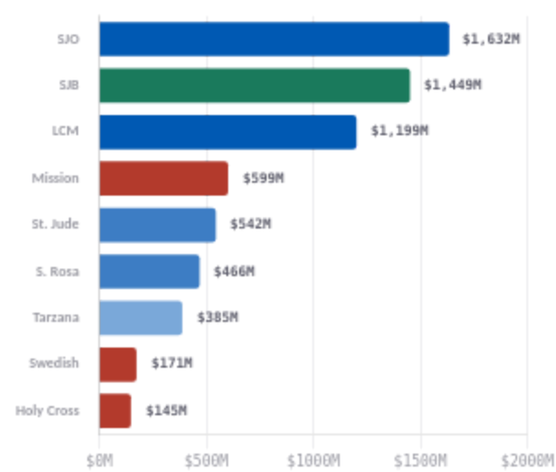
Conversion Rate by Ministry

% of prospects reaching Major Giving qualification



Qualified MG Net Worth by Ministry

Billions — qualified pipeline wealth



THE CORE INSIGHT

St. Joseph Burbank produces 30% of all system qualifications from just 7% of total prospects. It converts at 67.5% — nearly 5× the system average (14.9%). This is not a statistical anomaly; it's the result of specific, identifiable practices: heavy inpatient visit utilization (34% of contacts vs. 7.6% system-wide), direct physician referrals, and two development officers converting above 64%. Everything in this report flows from one question: how do we replicate this?

SECTION 03

The Four Result Multipliers

Data analysis reveals four specific levers that multiply results. Each is backed by actual conversion data — not theory.

MULTIPLIER 1 — INPATIENT VISITS

5.2×

Hospital Inpatient Visits convert at 40.8% vs. Letters at 7.9%. Each inpatient visit generates $4.8M in qualified MG net worth on average vs. $1.1M per letter. Yet visits represent only 7.6% of contacts while letters represent 49.6%. Shifting just 10% of letter volume to inpatient visits would produce an estimated 30–50 additional MG qualifications annually.

MULTIPLIER 2 — PHONE FOLLOW-UP

3.3×

Phone converts at 26.0% vs. Letters at 7.9% — a 3.3× improvement at moderate effort. Phone contacts generated $2.9B in qualified MG wealth, the highest absolute value of any method. The multi-touch progression from Letter → Phone → Visit compounds returns at each stage. Systematic phone follow-up on all letter outreach is the highest-efficiency intervention available.

MULTIPLIER 3 — DIRECT PHYSICIAN REFERRALS

3.0×

Clinician Referral (Direct) converts at 35.7% vs. List Review at 11.8%. Board Member referrals convert at 42.9%. The maturation path from passive list review to proactive referral is the single most valuable relationship progression. Dr. Pearre (41.7% conversion) and Dr. Kim (24.4%) prove this works — while four high-volume physicians with 0% conversion show what happens without it.

MULTIPLIER 4 — GRATITUDE-INTENSIVE DEPARTMENTS

2.0×

Genetics (86.7%), Gyn Oncology (52.6%), Infusion Therapy (27.0%), and Neurology (24.2%) all convert at 1.6–5.8× the system average. These departments share a common trait: extended, intensive clinical care that creates deep patient gratitude. Prioritizing clinician engagement in these departments produces disproportionate results per prospect contacted.

EXHIBIT B — CONTACT METHOD CONVERSION MATRIX

Contact Method × Results Output

Volume, conversion rate, and qualified net worth by introductory contact method

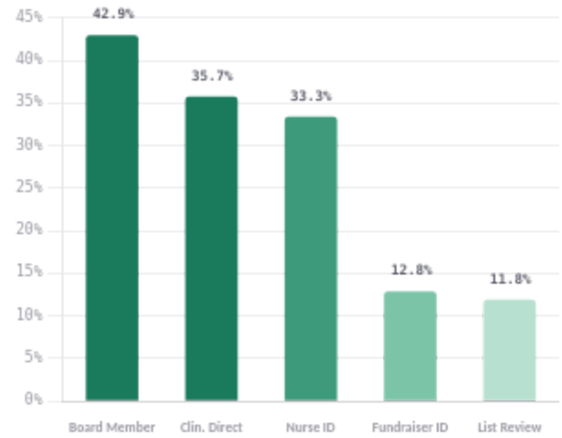
CONTACT METHOD	VOLUME	% OF TOTAL	MG QUALIFIED	MG RATE	MG NET WORTH	$ PER CONTACT
Hospital Inpatient Visit	223	7.6%	91	**40.8%**	$1,073M	$4.8M
Phone	723	24.5%	188	**26.0%**	$2,935M	$4.1M
Email	439	14.9%	48	**10.9%**	$550M	$1.3M
Letter	1,463	49.6%	116	**7.9%**	$1,579M	$1.1M

Volume vs. Conversion Rate

Bar = volume · Line = MG conversion rate

Referral Source Conversion

MG qualification rate by patient tracking source



SECTION 04

The Burbank Model — Anatomy of 67.5%

Burbank doesn't succeed through one tactic. It succeeds everywhere — every contact method, every referral source, every department. This section dissects the model that should be replicated system-wide.

67.5%
MG Conversion Rate
139 of 206 prospects qualify — 4.5× the system average of 14.9%

34%
Inpatient Visit Share
70 of 206 prospects contacted via inpatient visit — 4.5× the system average of 7.6%

67.1%
Visit Conversion Rate
Of SJB's inpatient visits, 67.1% convert to MG — well above the 40.8% system average

EXHIBIT C — BURBANK PERFORMANCE VS. SYSTEM

SJB vs. System Average

Side-by-side comparison across all dimensions

METRIC	SJB	SYSTEM	MULTIPLE
MG Conversion Rate	**67.5%**	14.9%	4.5×
Inpatient Visit Share	**34.0%**	7.6%	4.5×
Visit MG Conversion	**67.1%**	40.8%	1.6×
Phone MG Conversion	**58.0%**	26.0%	2.2×
Email MG Conversion	**96.0%**	10.9%	8.8×
Clinician Direct Rate	**91.0%**	35.7%	2.5×
List Review Rate	**56.0%**	11.8%	4.7×
Disqualification Rate	**20.4%**	21.7%	0.9×

Burbank doesn't just win on one metric — it wins on every metric. Its email conversion of 96% (vs. 10.9% system-wide) and its list review conversion of 56% (vs. 11.8%) suggest that the Burbank ecosystem pre-qualifies prospects at every stage.

Two development officers drive this performance:

THE OFFICERS BEHIND 67.5%

Brianne Bear converts at 64.7% (90 MG from 139 prospects) with only 67 phone calls — 0.7 calls per qualification. **Ani Kirikian converts at 73.1%** (49 MG from 67 prospects) with 50 calls. Compare this to Kristin Curren at LCM: 1,165 calls for 39 MG qualifications — 30 calls per qualification. The efficiency differential is **42×**. The question is what Bear and Kirikian do differently — and how to transfer that methodology.

WHAT MAKES IT REPLICABLE

Burbank's success is not about patient demographics or geography. It's an **operational environment**: high inpatient visit utilization, mature physician relationships that produce proactive referrals, and development officers who prioritize conversion quality over activity volume. Each of these is learnable and transferable.

SECTION 05

Where the Next 500 Qualifications Come From

The division doesn't need new prospects. It needs to convert the ones it already has. Here are the five largest pools of untapped results — quantified.

EXHIBIT D — GROWTH OPPORTUNITY SIZING

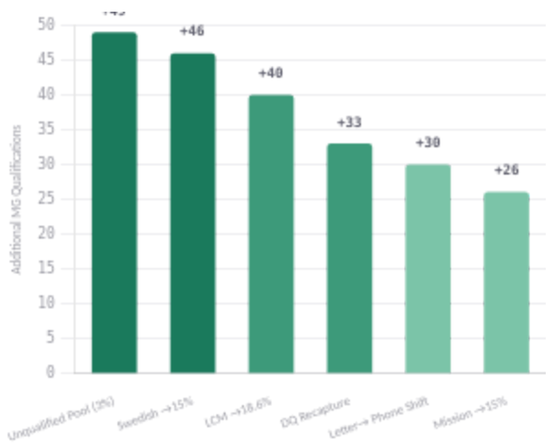
Incremental Qualification Projections

Based on reaching system median or proven benchmarks from existing pipeline

OPPORTUNITY	CURRENT	TARGET RATE	NEW MG QUAL	INCREMENTAL NW	BASIS
Mission → System Median (15%)	25 (7.3%)	15.0%	**+26**	$624M	Median rate
Swedish → System Median (15%)	9 (2.4%)	15.0%	**+46**	$874M	Median rate
LCM → SJO Rate (18.6%)	57 (10.9%)	18.6%	**+40**	$840M	SJO benchmark
10% Letter→Phone Shift	116 via letter	26.0%	**+30**	$540M	Phone rate
Unqualified Pool — 3% Activation	0 (1,621 prosp.)	3.0%	**+49**	$870M	Conservative
DQ Recapture — 5% Cycle	0 (663 disq.)	5.0%	**+33**	$325M	12-mo recapture
Total Incremental			**+224**	**$4.07B**	

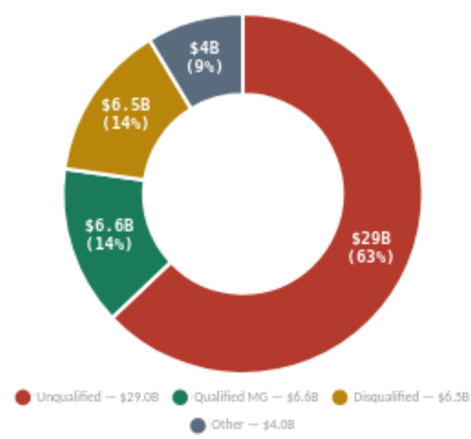
Incremental Qualifications by Source

Additional MG qualifications from each growth lever



The $29 Billion Unworked Pool

Stage distribution of pipeline net worth



THE SCALE OF THE OPPORTUNITY

224 additional major giving qualifications worth $4.07 billion in net worth — achievable from existing pipeline using proven benchmarks. This would nearly double the current 461 qualifications without identifying a single new prospect. The largest single opportunity: 1,621 unqualified prospects with $29.0 billion in net worth — higher average NW ($19.8M) than current qualified prospects ($16.3M) — where less than 1% have been contacted.

THE UNQUALIFIED POOL

These 1,621 prospects were identified by Prysm but never engaged by a development officer. Their aggregate net worth of $29.0B represents **63% of all identified pipeline wealth**. If the system converts just 3% of this pool, that produces $870M in new qualified pipeline — more than the total qualified output of 8 of the 11 ministries. The constraint is not identification. **The constraint is outreach capacity.**

SECTION 06

High-Performing Departments & Physicians

Results cluster in specific clinical settings and around specific physicians. These are the people and places to invest in.

EXHIBIT E — DEPARTMENT RESULTS LEADERS

Top Departments by Results Output

Ranked by MG qualification rate · Minimum 15 prospects

DEPARTMENT	PROSPECTS	MG RATE	MG NW	AVG NW
Genetics	15	**86.7%**	$255M	$17.9M
Gynecologic Oncology	19	**52.6%**	$63M	$7.4M
Infusion Therapy	37	**27.0%**	$245M	$19.6M
Neurology	66	**24.2%**	$170M	$13.6M
Rehabilitation	185	**18.4%**	$393M	$12.4M
Hematology & Oncology	169	**18.3%**	$265M	$13.3M
Cardiology	444	**17.6%**	$1,461M	$18.0M

DEPARTMENT PATTERN

The highest-converting departments share a common trait: **extended, intensive, gratitude-generating clinical care.** Genetics patients undergo multi-visit counseling; oncology patients receive sustained treatment; infusion therapy involves repeated visits over months. Each encounter deepens the gratitude bond that precedes philanthropic engagement. The strategic implication: invest clinician engagement resources in departments where the patient experience naturally creates giving propensity.

EXHIBIT F — PHYSICIAN RESULTS LEADERS

Top Converting Physicians

Ranked by MG conversion rate · Min 20 referrals

PHYSICIAN	REFS	MG	RATE
Pearre, Diana C	24	10	**41.7%**
Kim, Thomas C	41	10	**24.4%**
Kolski, Brian C	46	11	**23.9%**
Carpenter, Michele M	32	7	**21.9%**

High Volume, Zero Conversion

Physicians with 30+ referrals and 0% MG rate

PHYSICIAN	REFS	PIPELINE NW	MG
Wisniewski, Paul John	84	$24M	0
Pathi, Rama T	55	$56M	0
Mena, Raul R	38	$418M	0
Block, Edward J	32	$830M	0

THE ZERO-CONVERSION PHYSICIANS

209 referrals with $1.3B in pipeline wealth and zero qualifications. Dr. Block alone represents $830M in pipeline NW with no conversions. Understanding why these referrals don't convert — patient demographics, referral stage, downstream cultivation gaps — is a high-leverage investigation. Even converting 10% would add 21 qualifications.

SECTION 07

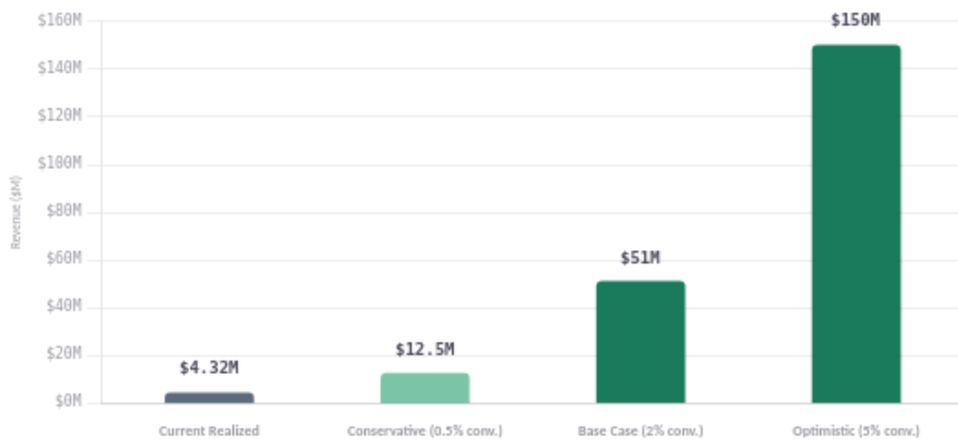
Revenue Projection — Three Scenarios

Modeled from current pipeline data, proven conversion benchmarks, and initial deployment results. The Base Case is anchored to Providence South's actual $4.32M realized revenue.

CONSERVATIVE	BASE CASE	OPTIMISTIC
$12.5M	**$51M**	**$150M+**
0.5% conversion · $25K avg gift	2% conversion · $50K avg gift	5% conversion · $100K avg gift
Current pace · Tier 1 only	Tier 3 activation · 2× outreach	Full activation + specialty playbooks
No additional staffing	Supported by proven benchmarks	System-wide physician maturation

Revenue Path from Current State

Conversion economics: small increases in contact rate produce outsized results



PROVEN BENCHMARK

$4.32M realized revenue from the initial Prysm deployment in Providence South, with a **448% engagement increase** and **305% first-time major gift increase**. These are audited results from the same platform, same division, same physician population. The Base Case of $51M over 3 years is realistic — it simply requires extending current proven practices across the full 3,098-prospect pipeline and activating the 1,621 unworked prospects.

THE MATH IS SIMPLE

Current state: **less than 1% of 100K+ identified patients contacted.** If 2% are contacted (~2,000 prospects enter funnel), and 15% qualify (the system median), that produces 300 new MG qualifications. At $50K average gift, that's $15M incremental per year. **Doubling outreach capacity doubles qualified pipeline — without identifying a single new patient.**

SECTION 08

90-Day Results Acceleration Plan

Six specific actions, ordered by expected impact, designed to generate measurable results within 90 days using existing pipeline and proven tactics.

1. Document the Burbank Playbook

Interview Bear & Kirikian. Map their exact workflow: inpatient visit timing, physician conversation framework, prospect prioritization criteria. Create a replicable SOP for distribution to all DOs. **Impact: Foundation for all other actions.**

2. Launch Phone Follow-Up Protocol

Implement systematic phone follow-up on all letter outreach within 14 days. Target: 100% of letter contacts receive at least one phone attempt. Based on 3.3× conversion lift from phone vs. letter. **Impact: +30–50 MG qualifications.**

3. Activate Unqualified Pool — Top 200

Prioritize the top 200 unqualified prospects by net worth for immediate DO outreach. Focus on the 5 largest hospital × specialty pools. Even 10% conversion yields 20 new MG qualifications from existing pipeline. **Impact: +20 qualifications, $396M pipeline.**

4. Swedish & Mission Intervention

Combined 711 prospects with $16.8B in pipeline wealth converting at 4.8%. Assign dedicated conversion improvement plan modeled on Burbank practices. Target: system median of 15%. **Impact: +72 MG qualifications, $1.5B pipeline.**

5. Physician Conversion Audit

Investigate the 4 zero-conversion physicians (209 referrals, $1.3B pipeline). Determine if it's a referral quality issue, cultivation gap, or screening mismatch. Begin proactive referral training with top-converting physicians. **Impact: +21 qualifications, $130M pipeline.**

6. DQ Recapture Cycle

Implement 12-month disqualification recapture protocol. 663 disqualified prospects with $6.5B in combined NW. At 5% recapture rate, recover 33 qualifications from prospects already in the system at zero acquisition cost. **Impact: +33 qualifications, $325M pipeline.**

90-DAY PHASED TIMELINE

DAYS 1–30 · ACTIVATE

Document + Deploy Foundation

Complete Burbank playbook documentation. Launch phone follow-up protocol across all ministries. Identify top 200 unqualified prospects for immediate outreach. Begin Swedish & Mission conversion audits. **Expected quick wins: 15–25 new MG qualifications from phone follow-up alone.**

DAYS 31–60 · SCALE

Expand Proven Tactics

Deploy Burbank playbook to St. Jude (25.5%) and Santa Rosa (20.3%) as first adopters. Begin systematic outreach to unqualified pool. Launch physician conversion audit. Implement DQ recapture cycle. **Expected output: 40–60 incremental MG qualifications.**

DAYS 61–90 · MEASURE

Benchmark + Optimize

Measure conversion rate changes at Mission and Swedish. Track DO productivity against Burbank benchmark (Bear: 0.7 calls/qualification). Report physician referral maturation rates. Size the next wave of unqualified pool activation. **Target: 100+ incremental MG qualifications in first 90 days.**

THE BOTTOM LINE

The pipeline exists. The wealth has been identified. The conversion model has been proven at Burbank. The division is currently converting 14.9% of identified prospects — and one ministry has proven that 67.5% is achievable. The gap between 14.9% and even 25% represents hundreds of additional major giving qualifications and billions in qualified pipeline. Every percentage point of improvement across $46.1B in pipeline wealth generates transformative results. The path to generating more results is not finding more prospects — it's converting the ones already in the system using the methods that are already working.